Sample limited to studies present in this tracker · Phase: all · Status: all · RT role: all · Disease: all · Country: all · Accrual: all · Source: all · Brief: all

Contents

Comprehensive · My trials · 7 studies · October 12, 2026 at 1:39 PM

Breast; Gynecologic 2

- NRG-BR009 (OFSET)

GU 3

- NRG-GU013 (High Five)
- NRG-GU015 (ARCHER)
- S2427 (BRIGHT)

Gynecologic 5

- NRG-GY037

Head and neck; Skin 6

- A222301 (HOPE)

NSCLC 7

- A081801

Breast; Gynecologic

NRG-BR009 (OFSET)

Phase 3 · NRG, NRG Oncology · recruiting · NCT05879926

A Phase III Adjuvant Trial Evaluating the Addition of Adjuvant Chemotherapy to Ovarian Function Suppression Plus Endocrine Therapy in Premenopausal Patients With pN0-1, ER-Positive/HER2-Negative Breast Cancer and an Oncotype Recurrence Score Less Than or Equal to 25 (OFSET)

Population. Premenopausal, resected ER-positive/HER2-negative breast cancer: pN0 with Oncotype RS 21-25, or RS 16-20 with high clinical risk (low grade and >3 cm, intermediate grade and >2 cm, or high grade and >1 cm); or pN1 with RS 0-25.

Step 1: Pre-entry. Registration. Patients qualifying by MammaPrint Low Risk/MP1 need central Oncotype DX RS before randomization; MammaPrint High Risk/MP2 are ineligible. Intended RT plan declared before pre-entry.

Stratification. Nodal/RS status (pN0 RS 16-25 vs pN1 RS 0-15 vs pN1 RS 16-25), intent to receive a CDK4/6 inhibitor (yes vs no), age (18-39 vs 40 or older).

Step 2: Randomize 1:1. Stratified permuted blocks.

Arm 1 Ovarian function suppression (GnRH agonist of investigator's choice, or bilateral oophorectomy) plus aromatase inhibitor for 5 years.

Arm 2 Adjuvant chemotherapy of investigator's choice (NCCN- or ESMO-listed regimen), then ovarian function suppression plus aromatase inhibitor for 5 years.

Radiotherapy (both arms). Whole breast irradiation required after lumpectomy (partial breast irradiation not permitted); post-mastectomy and regional nodal RT per protocol guidelines by nodal status. Allowed WBI regimens: 50 Gy/25, 42.56 Gy/16, 40 Gy/15, or 26 Gy/5, with optional sequential or concomitant boost. OFS plus AI preferably started before and given with RT, or within 4 weeks after RT.

Other therapy. CDK4/6 inhibitor (abemaciclib or other FDA-approved) and bone anti-resorptive therapy allowed per standard use.

Primary endpoint. Invasive breast cancer-free survival.

737 / 3960

GU

NRG-GU013 (High Five)

Phase 3 · NRG, NRG Oncology · recruiting · NCT05946213

The Phase III 'High Five Trial' Five Fraction Radiation For High-Risk Prostate Cancer

Disease. Pathologically proven adenocarcinoma of the prostate, meeting the high-risk definition in eligibility.

Stratify. Pelvic nodal radiotherapy yes vs no. ADT duration under 18 months vs over 18 months. Second-generation antiandrogen yes vs no. Intraprostatic microboost yes vs no.

Randomize 1:1. ADT of 6 to 36 months is required. Intended duration, type, and use of a second-generation antiandrogen are declared at registration.

Arm 1 Ultrahypofractionated SBRT in 5 fractions.

Arm 2 Conventional or moderate hypofractionation in 20 to 45 fractions.

Primary endpoint. Metastasis-free survival on conventional imaging.

1201 / 1500

NRG-GU015 (ARCHER)

Phase 3 · NRG, NRG Oncology · recruiting · NCT07097142

The Phase III Adaptive Radiation and Chemotherapy for Muscle Invasive Bladder Cancer Trial (ARCHER)

Disease. cT2-cT3N0 muscle-invasive urothelial carcinoma of the bladder.

Stratify. Clinical stage T2 vs T3. Planned radiosensitizer: cisplatin, gemcitabine, or mitomycin-C/5-FU. Neoadjuvant therapy yes vs no.

Randomize 1:1. Protocol treatment starts within 28 days after randomization. Arm 2 must use plan-of-the-day or online adaptive radiotherapy.

Arm 1 4 weeks of radiosensitizing chemotherapy plus hypofractionated radiotherapy, 55 Gy in 20 fractions.

Arm 2 4 weeks of radiosensitizing chemotherapy plus SBRT, 32.5 Gy in 5 fractions.

Primary question. Non-inferiority of ultra-hypofractionation versus hypofractionation for 3-year bladder-intact event-free survival. The margin is 10% (50% to 40%), corresponding to a hazard ratio below 1.32.

97 / 486

S2427 (BRIGHT)

Phase 2 · SWOG, National Cancer Institute (NCI) · recruiting · NCT07061964

Single Arm Phase II Study of Bladder Preservation With Immunoradiotherapy After a Clinically Meaningful Response to Neoadjuvant Therapy in Patients With Muscle Invasive Bladder Cancer (BRIGHT)

Muscle-invasive disease. Histologic cT2-T4aN0 urothelial carcinoma of the bladder.

Neoadjuvant therapy. Physician's choice, limited to the FDA-approved regimens in the eligibility criteria.

Response assessment. cT0 or cT1 on a repeat TURBT with systematic biopsies. That TURBT is within 90 days before registration.

Registration. Within 90 days after the last neoadjuvant dose.

Treatment. Bladder radiotherapy plus pembrolizumab for 1 year. Central radiation-plan approval is required before treatment starts. Radiotherapy is whole bladder only, 55 Gy in 20 fractions of 2.75 Gy, with no pelvic nodes and no tumor boost.

On-study assessment. Cystoscopy, cytology, and imaging.

Endpoint. 3-year bladder-intact event-free survival. Follow-up continues for 5 years after registration.

56 / 111

Gynecologic

NRG-GY037

Phase 3 · NRG, CCTG, National Cancer Institute (NCI) · recruiting · NCT07061977

A Phase III Study of Induction Pembrolizumab and Chemotherapy Followed by Chemoradiation and Pembrolizumab vs Chemoradiation and Pembrolizumab Both Followed by Pembrolizumab for High Risk Locally Advanced Cervical Cancer

Population. Newly diagnosed squamous cell carcinoma, adenocarcinoma, or adenosquamous carcinoma of the cervix, FIGO 2018 stage IIIA, IIIB, IIIC1, IIIC2, or IVA.

Stratification. FIGO stage III versus IVA, and presence or absence of para-aortic nodes (N2). Randomize 1:1.

Arm 1. Concurrent chemoradiation: weekly cisplatin 40 mg/m2 for 5 weeks, external-beam radiotherapy, brachytherapy, and pembrolizumab 200 mg every 3 weeks for 5 cycles, then pembrolizumab 400 mg every 6 weeks for 14 maintenance cycles.

Arm 2. Induction weekly carboplatin AUC 2 and paclitaxel 80 mg/m2 for 6 weeks, plus pembrolizumab 200 mg every 3 weeks for 2 cycles, then the same chemoradiation and pembrolizumab, then pembrolizumab 400 mg every 6 weeks for 15 maintenance cycles.

102 / 336

Head and neck; Skin

A222301 (HOPE)

Phase 3 · ALLIANCE, Alliance for Clinical Trials in Oncology, Alliance · recruiting · NCT06992427

High-Dose Prophylactic Gabapentin (HOPE) to Prevent Opioid Use for Oral Mucositis Pain During Head and Neck Chemoradiotherapy: A Phase III Clinical Trial

Arm A. Cisplatin-based chemoradiation, placebo, and the institution's standard treatment for mucositis.

Arm B. Cisplatin-based chemoradiation, prophylactic gabapentin titrated to 1200 mg three times a day, and the institution's standard treatment for mucositis.

Duration. About 10 weeks: a 12-day ramp up that may start any time from randomization through day 8 of chemoradiation, 7 weeks during radiotherapy, and a 12-day ramp down.

26 / 228

NSCLC

A081801

Phase 3 · ALLIANCE, National Cancer Institute (NCI) · recruiting · NCT04267848

Integration of Immunotherapy Into Adjuvant Therapy for Resected NSCLC: ALCHEMIST Chemo-IO (ACCIO)

Eligibility. Completely resected (R0) stage IIA, IIB, IIIA, or IIIB (T3-4N2) NSCLC per AJCC 8th edition, EGFR and ALK negative locally, registered 30-77 days after surgery. 1 cycle = 21 days.

Stratification. Histology (squamous vs non-squamous), local PD-L1 TPS (50% or higher vs below 50% or not evaluable), smoking status (never vs former/current), pathologic stage (IIA/IIB vs IIIA/IIIB T3-4N2).

Randomize. Since Update #07, randomization is to Arms B and C (the former Arm A, platinum doublet then observation, is closed). Platinum doublet is physician's choice: cisplatin/pemetrexed or carboplatin/pemetrexed for non-squamous, cisplatin/gemcitabine or carboplatin/paclitaxel for squamous.

Arm B (sequential) Platinum doublet x 4 cycles, +/- PORT, then pembrolizumab 400 mg IV every 6 weeks x 8 doses (16 cycles)

Arm C (concurrent) Platinum doublet x 4 cycles with pembrolizumab 400 mg on day 1 of cycles 1 and 3, +/- PORT, then pembrolizumab 400 mg every 6 weeks x 6 doses (12 cycles), 16 total cycles of pembrolizumab

PORT (optional). At treating physician discretion after the 4 chemotherapy cycles; recommended for occult pN2/N3, R1 margins, or extranodal extension, not for tumor size or pleural involvement alone. 50-54 Gy in 2 Gy (25-27 fractions) or 1.8 Gy (28-30 fractions). Starts 2-6 weeks after the last chemotherapy infusion and before continuation pembrolizumab, with no concurrent systemic therapy; pembrolizumab resumes 5-21 days after PORT.

Follow-up. Up to 10 years or until death. Primary endpoint DFS.

919 / 1210